

Case Report

Usefulness of homoeopathic medicine in treatment of primary infertility due to anti-müllerian hormone deficiency – A case report

Partha Pratim Pal¹, Souvik Dutta¹, Tania Chatterjee²

¹Dr. Anjali Chatterjee Regional Research Institute, Kolkata, West Bengal (An Institute under Central Council for Research in Homoeopathy), ²Central Council for Research in Homoeopathy, New Delhi, India.

*Corresponding author:

Dr. Tania Chatterjee,
Central Council for Research
in Homoeopathy, New Delhi,
India.

chatterjeetania8@gmail.com

Received: 22 October 2024
Accepted: 09 September 2025
Published: 16 October 2025

DOI
10.25259/JISH_93_2024

Quick Response Code:



ABSTRACT

The incidence of infertility in women is increasing amongst the Indian population, reflecting global trends. Anti-Müllerian Hormone (AMH) has become a vital marker for assessing ovarian reserve. There is growing interest in the factors influencing AMH levels, such as obesity, physical activity, smoking, alcohol consumption, as well as race, ethnicity and socioeconomic status. In certain cases, assisted reproductive technologies, such as *in vitro* fertilisation, may be suggested to improve the chances of conception. A 29-year-old woman visited the clinic, reporting difficulty in conceiving for 4 years. She was diagnosed with low ovarian reserve based on her low AMH level. The case was treated with homoeopathic medicine, and the patient conceived within 6 months, showing a single live intrauterine embryo with an average gestational age of 6 weeks, as confirmed using ultrasound. Assessment using the Modified Naranjo Criteria for Homeopathy at the final visit yielded a score of +8 on a scale of -6 to +13, suggesting a high probability that the patient's improvement was due to the homoeopathic treatment. This case report indicates that homoeopathic medicine can be effective for primary infertility associated with low AMH levels within a reasonable timeframe.

Keywords: Anti-Müllerian Hormone (AMH), Homoeopathy, Infertility, *Natrum muriaticum*

INTRODUCTION

The incidence of infertility in women is rising rapidly amongst the Indian population, mirroring trends seen globally.^[1] Over time, Anti-Müllerian Hormone (AMH) has emerged as a crucial marker for evaluating ovarian reserve. In addition, there is increased interest in exploring the factors that may affect AMH levels, including obesity, physical activity, smoking, alcohol consumption, as well as race, ethnicity and socioeconomic status. AMH is a dimeric glycoprotein that plays a crucial role in reproductive biology. It belongs to the transforming growth factor β (TGF- β) superfamily, which includes a wide range of proteins involved in various cellular processes. Notably, AMH features a single domain that shows homology to other members of the TGF- β superfamily. This homologous region is located in the 3' portion of the fifth exon and encodes the bioactive component of the AMH molecule. This segment is characterised by a glycine-cysteine-rich sequence, which is essential for the hormone's biological activity and functional interactions.^[2] AMH was identified in the 1940s by Alfred Jost, who outlined the hormone's role in embryonic

sexual differentiation.^[3] It has been established that AMH has a substantial effect on ovarian function, particularly in regulating follicle growth.^[4,5] This discovery has broadened the potential applications of AMH in gynaecology, extending its use from *in vitro* fertilisation (IVF) to the diagnosis of various ovarian diseases and cancers, as well as its prospective future applications.^[6] Many researchers advocate for the use of AMH as the most reliable biomarker for assessing ovarian reserve in patients. However, it is important to note that various genetic and environmental factors can influence both AMH gene expression and serum levels.^[7-10] Environmental factors known to affect AMH expression include vitamin D deficiency, obesity and smoking. In addition, genetic factors such as mutations in the *BRCA1*, *FMR1* and *MTHFR C677T* genes may also play a role.^[11] The European Society of Human Reproduction and Embryology consensus criteria identify AMH concentrations of <0.5–1.1 ng/mL or 3.57–7.85 pmol/L as indicators of poor ovarian reserve. However, some sources propose a higher cut-off value of 12 pmol/L for determining poor ovarian response.^[12] In women with primary ovarian insufficiency (POI), AMH concentrations are typically undetectable. Consequently, AMH levels are regarded as a key predictor of POI in women.^[13] Infertility in women due to decreased ovarian reserve has been coded as E28.8 under the International Classification of Diseases-10. Research has shown that in assisted reproductive technology (ART), serum AMH levels serve as a more reliable endocrine indicator of follicular response to ovarian stimulation than other commonly assessed markers, including follicle-stimulating hormone (FSH), oestradiol, inhibin B and patient age.^[14] Treating low AMH levels typically focuses on underlying issues affecting ovarian function. Options may include lifestyle modifications, such as improving diet and increasing exercise, as well as medical treatments like hormonal therapies to stimulate ovarian function. In some cases, ART IVF may be recommended to enhance the chances of conception. Individuals need to consult with a fertility specialist to determine the most appropriate approach based on their specific circumstances.^[6] Homoeopathy operates on the principle of using highly diluted substances to stimulate the body's self-healing abilities based on symptom similarity.^[15] In cases of primary infertility, homoeopathic medicine has proven to be effective in various studies.^[16,17] However, there are few documented clinical studies specifically evaluating homoeopathic treatments for primary infertility related to low AMH levels. The safety and efficacy of relying solely on Homoeopathy for managing this condition have not been established. This case report discusses the current scenario for the growing trend of female infertility in the Indian population context and explores how Homoeopathy may help to reduce both the economic burden and the side effects associated with conventional treatments of AMH-deficient primary infertility.

CASE REPORT

Patient information

A 29-year-old married woman along with her husband visited the clinic with complaints of failure to conceive.

Personal history

The couple, married since December 2017, began trying to conceive in 2020 but faced difficulties over the next 4 years.

Past treatment and medical history

After consulting a doctor in February 2022, they were advised to pursue IVF in August 2022, but they sought support through homoeopathic treatment, which they started in September 2022, in hopes of improving their chances of conception. The patient presented with a blood test report indicating an AMH level of 0.667 ng/mL, which is considered significantly low [Figure 1].

Past history

The patient had a history of recurrent urinary tract infections, occurring approximately twice a year over the past 7 years. In addition, she was hospitalised in 2013 due to dysentery.

Family history

As per family history concerns, her father experienced heart block while her mother had been diagnosed with depressive disorder.

Mental generals

The patient appeared to be quite sentimental and sensitive, expressing emotions easily. She reported that even minor contradictions often lead to headaches. In addition, she experienced episodes of weeping mania, characterised

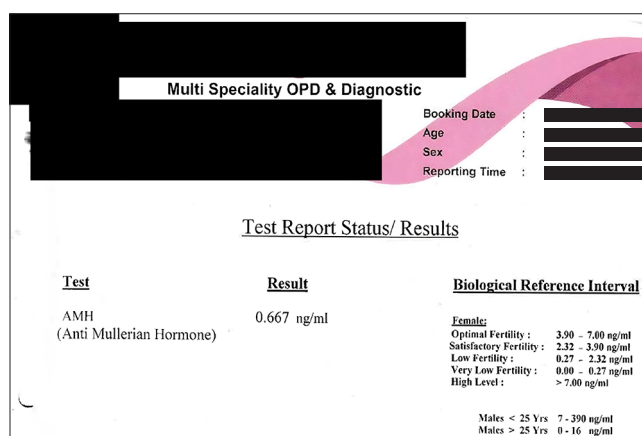


Figure 1: AMH test report.

by bouts of uncontrollable crying. Upon further inquiry, when she was asked about her response regarding receiving consolation from others, she conveyed that any consolation aggravates her complaints.

Physical general

The patient tended to catch colds frequently, with fluent coryza throughout the year. She described an empty and weak feeling in her stomach when she felt hungry. Furthermore, she had a noticeable intolerance to milk. Although her menstrual cycle was regular, the flow was notably scanty.

Clinical findings

No abnormal changes were found during superficial and deep palpation of the abdomen. The vitals were under normal range. No noticeable, significant changes were found during clinical examination.

Diagnostic assessment

The diagnosis was based on the laboratory report, which showed a low fertility level and provided indirect evidence that the patient had poor ovarian reserve [Figure 1].

Analysis of the case and repertorisation

Her complaints were carefully documented following a detailed case assessment, adhering to homoeopathic principles. Subsequently, after thorough analysis and evaluation [Table 1], the totality of symptoms was compiled, and the case was repertorised using Kent's repertory with the assistance of Homopath Classic M.D version 8 software.^[18] After repertorisation, *Natrum muriaticum* was

found to cover most symptoms and received the highest totality marks, followed by *Sepia*, *Pulsatilla*, and *Sulphur* [Figure 2].

Totality of symptoms

- Very sensitive and sentimental woman aged 29 years having a family history of depressive disorder who reported for treatment of her inability to conceive, probably due to low AMH level. Her menses were scanty despite being regular.
- Unable to bear even a minor contradiction that causes a headache.
- Weeping mania with bouts of uncontrolled crying.
- Complaints aggravated by consolation.
- Expresses her emotions easily.
- Significant intolerance to milk and a tendency to develop rhinorrhoea throughout the year.
- Emptiness and weakness in the stomach when hungry.

Final remedy selection

A sensitive and sentimental woman having intolerance to contradiction, causing headache and having bouts of involuntary weeping whereby complaints are <by consolation, is indicating towards the selection of *Natrum muriaticum*. Other dispositional features are also within the curative power of *Natrum muriaticum*. Hence, the selection of the remedy in this case of infertility was *Natrum muriaticum*.

Therapeutic intervention

Based on the totality of symptoms, individualisation, repertorisation, and referral to the *Materia Medica*,^[19,20] *Natrum muriaticum* 1 M was prescribed in 30 no globules

Table 1: Analysis and evaluation of symptoms, conversion of symptoms to rubrics.

Evaluation order	Symptoms	Characterised by	Evaluation reason	Rubric (Dr. J. T. Kent's repertory)
1	Sentimental and sensitive	- Expressing emotions easily - Contradictions - Headache - Weeping mania, having bouts of uncontrolled crying - Complaints<consolation	Singular mental disposition	- Mind, Weeping, involuntary - Mind, Contradiction, is intolerance of - Mind, Consolation agg.
2	Intolerance to milk		Singular disposition	Generalities, Food, Milk agg.
3	Headache	Ailments from contradiction	Peculiar disposition	Head, Pain, contradiction after
4	Menses scanty	Despite a regular menstrual cycle	Uncommon symptom	Genitalia, Female, Menses, scanty
5	Tendency to catch colds frequently	Fluent coryza throughout the year	Peculiar disposition	- Generalities, Cold tendency to take - Nose, Coryza, discharge with (fluent)
6	Empty and weak feeling in her stomach	If hungry	Peculiar symptom	Generalities, Hunger from

Remedy	Nat-m	Sep	Puls	Sulph	Phos	Nit-ac	Alum	Ign	Lyc	Con	Kali-c	Nux-v
Totally	15	14	13	13	12	11	10	10	10	10	10	10
Symptoms Covered	6	5	5	5	7	4	5	5	5	4	4	4
Kingdom												
[Kent] [Mind]WEEPING, TEARFUL MOOD, ETC.:Involuntary: (23)	3	3	3		1		2	3				
[Kent] [Generalities]FOOD:Milk :Agg: (57)	2	3	2	3	2	3	2	1	2	3	2	2
[Kent] [Generalities]COLD :Tendency to take: (93)	3	3	2	2	2	3	3	1	3	2	3	3
[Kent] [Generalities]FAINTNESS,FAINTING:Hunger,from: (2)				2	2							
[Kent] [Head]PAIN,HEADACHE IN GENERAL:Contradiction,after: (9)	1				1				1			
[Kent] [Genitalia female]MENSES:Scanty: (101)	3	3	3	3	3	2	2	2	2	3	3	2
[Kent] [Nose]CORYZA:Discharge,with (fluent): (160)	3	2	3	3	1	3	1	3	2	2	2	3

Figure 2: Repertorial sheet.

Table 2: Follow-up		
Visits	Last menstrual period	Prescription with justification
Visit 1 22 September, 2022	28 September, 2022 Flow=3 days	<i>Natrum muriaticum</i> 1 M/2 doses OD for 2 days. • Weeping mood with consolation aggravation • Headache contradiction after • Fluent nasal discharge • Weak feeling on an empty stomach • Scanty menses • Easily catches a cold • Milk intolerance.
Visit 2 27 October, 2022	30 October, 2022 Flow=3 days	• Patient did not experience any headache, fluent nasal discharge in the past 1 month • She was in a better phase to deal with her weeping mood frequently • She was not complaining of any weak sensation in her stomach • Overall, the patient experienced a sense of well-being. Hence, a placebo was prescribed for 30 days, to be taken once daily.
Visit 3 28 November, 2022	25 November, 2022 Flow=3 days	• The patient experienced a headache twice after a few conflicts in conversation • She cried alone as an outburst of her emotion • She started feeling emptiness in her stomach, while she used to feel hungry. Considering the persistence of these symptoms, <i>Natrum muriaticum</i> 10M, single dose, was prescribed, and the patient was asked to take the medicine early in the morning on an empty stomach.
Visit 4 03 January, 2023	29 December, 2023 Flow=3 days	• The patient did not complain of any headache, nasal discharge, or weak feeling in the stomach. Hence, placebo 200 was prescribed for 30 days, to be taken once daily.
Visit 5 09 February, 2023	29 December, 2023	Advice for urine HCG test
OD: Once a day; HCG: Human chorionic gonadotropin		

manufactured by a pharmaceutical company certified under good manufacturing practice. She was instructed to take four globules of the medicine early in the morning on an empty stomach for 2 days, followed by a placebo for 1 month.

Follow-up and outcomes

On the 3rd visit, the patient was prescribed *Natrum muriaticum* 10 M single dose followed by placebo [Table 2]. On the 5th visit, the patient was advised to undergo a urine

Table 3: Assessment of treatment outcomes in Homoeopathy using the modified *Naranjo criteria* (MONARCH)

Domains	Monarch	Answers of the patient	Scores
1.	Was there an improvement in the main symptom or condition for which the homeopathic medicine was prescribed?	Yes	+2
2.	Did the clinical improvement occur within a plausible time frame relative to the medicine intake?	Yes	+1
3.	Was there a homeopathic aggravation of symptoms?	No	0
4.	Did the effect encompass more than the main symptom or condition (i.e., were other symptoms, not related to the main presenting complaint, improved or changed)?	Yes	+1
5.	Did overall well-being improve? (suggest using a validated scale or mention changes in physical, emotional and behavioural elements)	Yes	+1
6.	(A) <i>Direction of cure</i> : Did some symptoms improve in the opposite order of the development of symptoms of the disease?	Not sure	0
	(B) <i>Direction of cure</i> : did at least one of the following aspects apply to the order of improvement of symptoms: – from organs of more importance to those of less importance? – from deeper to more superficial aspects of the individual? – from the top downwards?	Not sure	0
7.	Did ‘old symptoms’ (defined as non-seasonal and non-cyclical symptoms that were previously thought to have resolved) reappear temporarily during the course of improvement?	No	0
8.	Are there alternative causes (i.e., other than the medicine) that – with a high probability – could have produced the improvement? (Consider known course of disease, other forms of treatment and other clinically relevant interventions)	No	+1
9.	Was the health improvement confirmed by any objective evidence? (e.g. investigations, clinical examination, etc.)	Yes	+2
10.	Did repeat dosing, if conducted, create similar clinical improvement?	Not sure	0
Total score: +8 (Note: Maximum score: +13, minimum score: –6)			

human chorionic gonadotropin (HCG) test, which was positive [Figure 3]. An ultrasound sonography (USG) of the lower abdomen on 13 February 2024 revealed a single live intrauterine embryo with an average gestational age of 6 weeks and 3 days [Figure 4].

DISCUSSION

The rising incidence of infertility in women amongst the Indian population reflects a global trend that has garnered significant attention in recent years. A key indicator in this context is AMH, which serves as an essential marker for assessing ovarian reserve.^[2] As researchers delve into the factors influencing AMH levels, a range of elements has come to light, including obesity, physical activity, smoking, alcohol consumption and demographic factors such as race, ethnicity and socioeconomic status.^[11]

Managing low AMH levels usually involves addressing the underlying factors affecting ovarian function. This may include lifestyle modifications, such as enhancing diet and increasing physical activity, along with medical interventions like hormonal therapies to stimulate ovarian activity. In certain situations, ART such as IVF may be suggested to improve the likelihood of conception.^[6] It is also hypothesised

that vitamin D has a direct effect on AMH production, suggesting that patients with higher levels of vitamin D may be able to preserve their ovarian reserve for a longer period.^[21] Studies suggest that vitamin D acts as a positive regulator of AMH production.^[22] Other reports indicate that vitamin D levels are unrelated to ovarian reserve or ovarian response following ovarian stimulation.^[23,24] Homoeopathy aims to activate the body's natural healing abilities and strengthen the immune system through the use of low doses of homoeopathic remedies. It emphasises treating the individual as a whole and addressing the root causes of illness rather than merely alleviating symptoms. Homoeopathy can be effective for infertility related to psychological issues, ovulation disorders, sperm abnormalities and cases of unknown origin.^[25,26] Meanwhile, Homoeopathy has been shown to effectively enhance patients' overall health, improve sperm quality and regulate hormone levels.^[27] Parveen and Bhaumik^[28] employed individualised Homoeopathy to help an infertile patient with endometriosis, fallopian tube abnormalities and insufficient ovarian reserve successfully deliver a healthy newborn, indicating that Homoeopathy can have a positive impact on infertility. None of these trials addresses the role of AMH levels in infertility and the application of Homoeopathy in this context. In this case report, a patient with a low AMH level became pregnant with

DR. NAME : [REDACTED] AGE: [REDACTED] SEX: [REDACTED]
DATE: [REDACTED]

REPORT OF HORMONE ANALYSIS BY IMMUNOCHROMATOGRAPHY

Urinary hcG	Result	: POSITIVE
Qualitative Test	Sensitivity	: 25 mIU/ml

• SUGGESTED CLINICAL CORRELATION.

Figure 3: Urine HCG test report.

Age/Sex : [REDACTED]
Date : [REDACTED]

Part Exam : USG OF OBSTETRIC

REPORT OF ULTRASONOGRAPHY
THANKS FOR REFERRING THE PATIENT FOR ULTRASONOGRAPHY

LMP:- 29.12.2022 GA ACCORDING TO LMP: 06 W 4 D EDD BY LMP: 05.10.2023

URINARY: Is adequately distended & contour is normal. Wall thickness is normal.
BLADDER: No calculus is seen.

UTERUS: - Gravid & bulky uterus showing single intrauterine gestational sac. Single live embryo is seen within the gestational sac.
CRL of embryo measuring 0.5 cm which corresponds to 06 weeks 03 days.

Sonological E.D.D. - 06.10.2023

F.H.R.:- 125 bpm.

PLACENTA: Placenta is developing.

Internal os: - Closed.

P.O.D.:- Clear.

Impression: U.S.G. reveals:
• Single live intrauterine embryo with average gestational age 06 weeks 03 days maturity.

Figure 4: USG of lower abdomen.

a viable foetus at 6 weeks and 3 days after only 6 months. In this case, the total outcome score according to the modified Naranjo criteria^[29] was +8, approaching the maximum score of 13 [Table 3]. This clearly indicates the strong causal

link attributed to the homeopathic treatment for primary infertility and the outcome. This shows a positive effect of homeopathic medicinal treatment upon low levels of AMH-associated with primary infertility. Therefore, there is a need for well-designed research studies to establish the efficacy of Homoeopathy in primary infertility associated with low serum AMH levels. The probable limitations of the study were the non-availability of luteinising hormone (LH), FSH and oestradiol evaluation reports on the 3rd day of menses, as the patient was unwilling to undergo this test repeatedly. Due to the positive HCG report and USG of the lower abdomen, post-treatment AMH advice was neglected by the patient.

CONCLUSION

This case report suggests homoeopathic treatment as a promising complementary and alternative medicinal therapy and emphasises the individualisation and the need for repertorisation in Homoeopathic prescription. This case shows a positive role of Homoeopathy in the treatment of low AMH level induced primary infertility. Randomised control trials are suggested with proper methodological rigour to establish the efficacy of Homoeopathy.

Ethical approval: Institutional review board approval is not required.

Declaration of patient consent: The authors certify that they have obtained all appropriate patient consent.

Financial support and sponsorship: Nil.

Conflicts of interest: There are no conflicts of interest.

Use of artificial intelligence (AI)-assisted technology for manuscript preparation: The authors confirm that there was no use of artificial intelligence (AI)-assisted technology for assisting in the writing or editing of the manuscript and no images were manipulated using AI.

REFERENCES

- Kundu S, Ali B, Dhillon P. Surging trends of infertility and its behavioural determinants in India. *PLoS One* 2023;18:e0289096.
- Josso N, Di Clemente N, Gouédard L. Anti-Müllerian hormone and its receptors. *Mol Cell Endocrinol* 2001;179:25-32.
- Josso N, Picard JY, Rey R, di Clemente N. Testicular anti-Müllerian hormone: History, genetics, regulation and clinical applications. *Pediatr Endocrinol Rev* 2006;3:347-58.
- Rey R, Sabourin JC, Venara M, Long WQ, Jaubert F, Zeller WP, *et al.* Anti-Müllerian hormone is a specific marker of sertoli- and granulosa-cell origin in gonadal tumors. *Hum Pathol* 2000;31:1202-8.
- Visser JA, de Jong FH, Laven JS, Themmen AP. Anti-Müllerian hormone: A new marker for ovarian function. *Reproduction* 2006;131:1-9.
- Bedenk J, Vrtačnik-Bokal E, Virant-Klun I. The role of anti-Müllerian hormone (AMH) in ovarian disease and infertility. *J Assist Reprod Genet* 2020;37:89-100.
- Abdel Hafez FF, Tang Y, Hassan MH, Saleem TH. Assessment

- of anti-Müllerian hormone (AMH) levels in a pilot cohort of peripubertal females: Correlation with sex maturity rating (SMR). *Middle East Fertil Soc J* 2018;23:278-80.
8. Anderson RA. What does anti-Müllerian hormone tell you about ovarian function? *Clin Endocrinol (Oxf)* 2012;77:652-5.
 9. Sun H, Mao H, Cai J, Zhao Y. Research progress on anti-müllerian hormone clinical applications and immunoassay development. *Front Lab Med* 2018;2:14-8.
 10. Nair S, Slaughter JC, Terry JG, Appiah D, Ebong I, Wang E, *et al.* Anti-müllerian hormone (AMH) is associated with natural menopause in a population-based sample: The CARDIA Women's Study. *Maturitas* 2015;81:493-8.
 11. Shahrokhi SZ, Kazerouni F, Ghaffari F. Anti-Müllerian hormone: Genetic and environmental effects. *Clin Chim Acta* 2018;476:123-9.
 12. Alson SS, Bungum LJ, Giwercman A, Henic E. Anti-Müllerian hormone levels are associated with live birth rates in ART, but the predictive ability of anti-Müllerian hormone is modest. *Eur J Obstet Gynecol Reprod Biol* 2018;225:199-204.
 13. Knauff EA, Eijkemans MJ, Lambalk CB, ten Kate-Booij MJ, Hoek A, Beerendonk CC, *et al.* Anti-Müllerian hormone, inhibin B, and antral follicle count in young women with ovarian failure. *J Clin Endocrinol Metab* 2009;94:786-92.
 14. Arce JC, Andersen AN, Fernández-Sánchez M, Visnova H, Bosch E, García-Velasco JA, *et al.* Ovarian response to recombinant human follicle-stimulating hormone: A randomized, anti-Müllerian hormone-stratified, dose-response trial in women undergoing *in vitro* fertilization/intracytoplasmic sperm injection. *Fertil Steril* 2014;102:1633-40.e5.
 15. Teixeira MZ. Homeopathic use of modern medicines: Utilisation of the curative rebound effect. *Med Hypotheses* 2003;60:276-83.
 16. Lobo A, D'cunha P, Lobo B. Effectiveness of homeopathic treatment in female infertility. *Reprod Med Int* 2018;1:008.
 17. Pandit RK. An evidence-based case report of female infertility treated with individualised homeopathic medicine. *Homeopathic Links* 2024;37:50-3.
 18. Shah J. *Homopath classic-homeopathic software. Version 8.0 premium.* Mumbai: Homopath; 2005.
 19. Boericke W. *New manual of homeopathic materia medica with repertory.* 9th ed. New Delhi: B Jain Publishers Pvt Ltd.; 2013.
 20. Hering C. *The guiding symptoms of our materia medica.* Vol. 7. Reprint edition. New Delhi: B Jain Publishers Pvt Ltd.; 1986. p. 556.
 21. Grzechocinska B, Dabrowski FA, Cyganek A, Wielgos M. The role of vitamin D in impaired fertility treatment. *Neuro Endocrinol Lett* 2013;34:756-62.
 22. Dennis NA, Houghton LA, Jones GT, van Rij AM, Morgan K, McLennan IS. The level of serum anti-Müllerian hormone correlates with vitamin D status in men and women but not in boys. *J Clin Endocrinol Metab* 2012;97:2450-5.
 23. Fabris AM, Cruz M, Iglesias C, Pacheco A, Patel A, Patel J, *et al.* Impact of vitamin D levels on ovarian reserve and ovarian response to ovarian stimulation in oocyte donors. *Reprod Biomed Online* 2017;35:139-44.
 24. Bednarska-Czerwińska A, Olszak-Wąsik K, Olejek A, Czerwiński M, Tukiendorf AA. Vitamin D and Anti-Müllerian hormone levels in infertility treatment: The change-point problem. *Nutrients* 2019;11:1053.
 25. Besnard-Charvet C. Accompany the medically assisted procreation. *The Revue d'Homeopathy* 2014;5:120-4.
 26. Balaji Deekshitulu PV. Homeopathic treatment for female infertility. *Int Res J Obstet Gynecol* 2020;3:1-6.
 27. Feng J, Wang J, Zhang Y, Zhang Y, Jia L, Zhang D, *et al.* The efficacy of complementary and alternative medicine in the treatment of female infertility. *Evid Based Complement Alternat Med* 2021;6634309.
 28. Parveen S, Bhaumik H. Effect of individualised homoeopathy in the treatment of infertility. *Indian J Res Homoeopathy* 2018;12:231-9.
 29. Lamba CD, Gupta VK, van Haselen R, Rutten L, Mahajan N, Molla AM, *et al.* Evaluation of the modified naranjo criteria for assessing causal attribution of clinical outcome to homeopathic intervention as presented in case reports. *Homeopathy* 2020;109:191-7.

How to cite this article: Pal PP, Dutta S, Chatterjee T. Usefulness of homeopathic medicine in treatment of primary infertility due to anti-müllerian hormone deficiency – A case report. *J Intgr Stand Homoeopathy*. 2025;8:94-100. doi: 10.25259/JISH_93_2024